KENYA MEDICAL TRAINING COLLEGE
FACULTY OF NURSING
DEPARTMENT OF NURSING

FINAL QUALIFYING EXAMINATION
FOR
DIPLOMA IN COMMUNITY HEALTH NURSING

PAPER: MIDWIFERY

DATE: 23rd JULY, 2024	**TIME: 3 HOURS (9:00 AM – 12 NOON)**

INSTRUCTIONS

1. This paper consists of:
   - Section 1 (40 Multiple Choice Questions)
   - Section 2 (7 Short Answer Questions)
   - Section 3 (1 Long Answer Question)
2. Attempt **ALL** Questions
3. ALL Multiple choice questions MUST be answered in **CAPITAL LETTERS**
4. Write your **EXAMINATION NUMBER** on all the answer sheets provided and on the question paper.
5. No other writing should be done on the question paper except your Examination Number.
6. Ensure that all examination answer scripts are handed in at the end of the examination
7. Ensure you sign the examination register provided

EXAM

MIDWIFERY

SECTION 1: MULTIPLE CHOICE QUESTIONS **(40 MARKS)**

1. During the pre-historic age midwifery was practiced by: -
   A. Trained midwives
   B. Ordinary trained nurses
   C. Uneducated but experienced women
   D. Midwives and doctors.
2. You are teaching anatomy and physiology to newly admitted nursing students. The features that are true about the android pelvis that you would share with them include:
   A. Rounded brim, narrow fore pelvis, Ischial spines blunt.
   B. Fore pelvis wide, sciatic notch wide, sub-pubic angle more than 90^0.
   C. Sciatic notch rounded, brim rounded, ischial spines
   D. Rounded brim, side walls are straight, fore pelvis generous.
3. You are recording your findings on the partograph. An indicator of good progress in established labor would be:
   A. Cervical dilatation to the Right of the Alert Line
   B. Cervical dilatation to the Right of the Action Line
   C. Foetal descend of 5/5 at 8 cm dilatation
   D. Two uterine contractions every 10 min lasting 10 sec
4. According to the schedule for Targeted Postnatal Care, the second visit typically occurs within
   A. 1 to 2 days
   B. 1 to 2 weeks
   C. 1 to 2 months
   D. 1 to 2 years
5. You are reviewing a pregnant woman and indicate she is para 1+1. The universal meaning of parity is
   A. The number of times a woman has been pregnant.
   B. A woman who has never given birth at all.
   C. The number of pregnancies a woman has delivered at ter.
   D. A woman who has delivered more than five (5) times.
6. In midwifery, the pelvic joints that are particularly relevant include:
   A. One symphysis pubis, two sacroiliac joints, two sacrococcygeal joint.
   B. One symphysis pubis, two sacroiliac joints, one sacrococcygeal joint.
   C. One symphysis pubis, one sacrococcygeal joint, one sacral joint.
   D. Sacral joint, one sacroiliac joint, one symphysis pubis
7. The diameters of the pelvic brim are important in obstetrics as they determine the passage through which the baby must descend during childbirth. The diameters are:
   A. Anteroposterior diameter, oblique diameter, transverse diameter.
   B. Longitudinal diameter, transverse diameter, oblique diameter.
   C. Oblique diameter, longitudinal diameter, anteroposterior diameter.
   D. Longitudinal, pelvic brim, transverse
8. During fetal development, the organs formed from the ectoderm layer include:
   A. Mucous membranes and glands
   B. Bones, muscles, the heart and blood vessels
   C. Skin and the nervous system
   D. Nervous system and mucous membranes

9. The membranous space on the fetal skull that separates the frontal bones from the two parietal bones is called the:
   A. Frontal
   B. Coronal
   C. Sigittal
   D. Lambdoidal
10. During pregnancy, several physiological changes occur in the alimentary system to support the needs of the growing fetus and ensure proper nourishment. These changes include:
   A. Increased Oesophageal peristalsis, accompanied by gastric reflux
   B. Decreased Gastrointestinal motility
   C. Increased gastrointestinal motility
   D. Gastric reflux is more prevalent in early pregnancy
11. During normal labor, which consists of three stages:
   A. The foetus, placenta, and membranes are expelled through the birth canal.
   B. The Foetus is expelled through birth canal at full dilatation of cervix.
   C. A series of passive movements of the foetus through the birth canal.
   D. A foetus at term, in vertex presentation, is spontaneously expelled within 18 hours without complications to mother or baby.
12. In false labor, several characteristics differentiate it from true labor contractions. The contractions are:
   A. Painful, regular, last 40 to 60 seconds.
   B. Painful, irregular, last 2 to 3 minutes.
   C. Painful, regular, increase in frequency and strength.
   D. Painful, regular, are accompanied by backache.
13. Moulding in the context of childbirth refers to the shaping or elongation of the fetal skull bones to allow passage through the birth canal during labor. It is identified by detecting changes in:
   A. Swelling of presenting part
   B. Position of sutures
   C. Overlapping of skull bones
   D. Position of occiput
14. During labour, uterine muscles contract and retain the shortening. This is: -
   A. Fundal dominance
   B. Polarity
   C. Retraction
   D. Retraction ring
15. The sinciput, face and chin sweep perineum as nape of neck pivots lower border of symphysis pubis. This is: -
   A. Extension of the head
   B. External rotation of the head
   C. Lateral flexion of the head
   D. Internal rotation of the head
16. During daily examination, genitalia of a newborn is checked for: -
   A. Pseudo-menstruation in the female
   B. Epispadias in the male
   C. Pseudohermaphroditism
   D. Labia Majora that does not cover the labia minora

17. Newborn holds object firmly enough to be momentarily lifted off the cot. This is referred to as:
    A. Moro reflex
    B. Tonic neck reflex
    C. Rooting reflex
    D. Grasping reflex
18. Probable signs for 2nd stage of labour include;
    A. Expulsive uterine contractions, no cervix felt
    B. Trickle of blood, expulsive uterine contractions
    C. Gaping of the vulva, no cervix felt
    D. No cervix felt, gaping of the anus
19. A woman who is at 32 weeks gestation has had ruptured membranes for 26 hours. As a midwife, you would assess the woman for which of the following manifestation
    A. Proteinuria
    B. Dependent edema
    C. Constipation
    D. Elevated temperature
20. The most common causative organism of ophthalmia neonatorum is: -
    A. Candida albicans.
    B. Neisseria gonorrhea.
    C. Beta- haemolytic streptococcus
    D. Chlamydia-trachomatis
21. During the first few days of life the type of jaundice the neonates get as a normal process is called: -
    A. Obstructive jaundice
    B. Haemolytic jaundice
    C. Neonatal jaundice
    D. Physiologic jaundice.
22. The first step you would take to prevent hypothermia in a neonate is to keep baby warm by: -
    A. Transferring baby if necessary.
    B. Skin to skin contact with mother.
    C. Breastfeeding the baby immediately.
    D. Keep clothed baby under heat source.
23. Preterm baby refers to a baby born
    A. Before 37 completed weeks of pregnancy
    B. Between 24 weeks and 28 weeks.
    C. Between 28 weeks to 34 weeks of pregnancy.
    D. Before 36 weeks gestation.
24. Predisposing factors to asphyxia neonatorum include: -
    A. Cord prolapse, low immunity, APH
    B. Low immunity, primigravida, birth injuries.
    C. Prematurity, cord prolapse, APH.
    D. Primigravida, low immunity, cord prolapse.
25. Epidural anaesthesia given to relief labor pains is contra-indicated in women with:
    A. Hypovolemia, multiple pregnancy, pre-eclampsia
    B. Local infection, hypovolemia, coagulation
    C. Breech presentation, multiple pregnancy, pre-eclampsia
    D. Multiple pregnancy, coagulation disorders, breech presentation

26. A nonpharmacologic strategy of pain relief during labour, in which the woman's awareness of the external environment is reduced is known as: -
    A. Acupressure
    B. Transcutaneous electrical nerve stimulation
    C. Aromatherapy
    D. Hypnosis
27. Risks associated with teenage pregnancy during the antenatal period include: -
    A. Malnutrition, rejection, prolonged labour
    B. Post-partum haemorrhage, prolonged labour, pre-eclampsia
    C. STIs/HIV/AIDs, anaemia, pre-eclampsia
    D. Post-partum haemorrhage, intra-uterine fetal death, malnutrition
28. Hyperemesis gravidarum is associated with the following conditions:
    A. Infective hepatitis, hydatid form mole, pyelonephritis
    B. Multiple pregnancy, first pregnancies, hydatid form mole
    C. Hydatid form mole, infective hepatitis, malaria
    D. Pyelonephritis, multiple pregnancy, meningitis
29. The following criteria is used to initiate ART in women who are HIV positive and pregnant
    A. WHO stage 3 and 4.
    B. CD4 < 350
    C. On the same day HIV diagnosis is made
    D. At a gestational period of 28 weeks
30. A baby born to a diabetic mother should be observed closely for at least for 48 hours to detect and treat the following common complications.
    A. Hyperglycemia, Respiratory Distress Syndrome, Asphyxia
    B. Asphyxia, Birth Injury, Hypoglycemia
    C. Hypoglycemia, Respiratory Distress Syndrome, Jaundice
    D. Jaundice, Hyperglycemia, Respiratory Distress Syndrome
31. An institution that ensures that children are placed in suitable homes in line with the best interest of the child is:
    A. The court
    B. The police station
    C. An adoption agency
    D. The county hospital
32. The structures involved in third degree perineal tears are;
    A. Injury to perineum involving perineal muscles but not involving the anal sphincter
    B. Injury to the perineum involving the anal sphincter complex
    C. Injury to skin only
    D. Injury to the anal sphincter including the rectal mucosa
33. In frank breech;
    A. Thighs are flexed but the legs are extended to the abdomen
    B. Legs are flexed but thighs are extended
    C. Both thighs and legs are extended
    D. Both thighs and legs are extended and feet tucked behind the buttocks
34. The manoeuvre used in delivering breech with extended legs is;
    A. Pinard manouvre
    B. Burn-Marshals manouvre
    C. Maureceu- Smellie –Veit manouvre
    D. Lovset manouvre

35. The presenting diameter in face presentation is;
    A. Sub-mento vertical diameter
    B. Mento-vertical diameter
    C. Occipito-frontal diameter
    D. Sub-mento bregmatic
36. The characteristic of third degree inversion of the uterus is;
    A. The inverted fundus extends through the cervix but remains within the vagina.
    B. The inverted fundus extends to, but not through the cervix.
    C. The inverted fundus extends outside the vagina
    D. Both the vagina and uterus are inverted
37. The best position to place a patient during correction of inversion of the uterus is;
    A. Exaggerated Sims position
    B. Knee chest position
    C. Lithotomy position
    D. Trendelenburg position
38. The outcome of labour in short internal rotation in occipito posterior position is;
    A. Normal delivery
    B. Deep transverse arrest
    C. Brow presentation
    D. Face to pubis
39. The characteristics of monozygotic twins include that they:
    A. Have two amnions, two chorions, fertilized by one spermatozoa
    B. Develop from one ovum, same sex, two placentae
    C. Have one amnion, different sexes, one placenta
    D. Are of same sex, develop from one ovum, have one placenta
40. Mrs. R is rhesus negative. Under what circumstance would she be eligible for RH (D) immune globulin (RhIG)
    A. If she had multiple pregnancy
    B. If her fetus was found to be Rhesus negative
    C. If her fetus heartbeat was tachycardic
    D. If her fetus blood type was found to be Rhesus positive

SECTION 2: SHORT ANSWER QUESTIONS (40 MARKS)

41. Focused Antenatal Care (FANC) is a systematic approach to providing healthcare and monitoring during pregnancy. State four (4) objectives of Focused Antenatal Care. (4 marks)
42. Puerperium refers to the period immediately following childbirth during which the mother's body undergoes physiological changes as it returns to its non-pregnant state. Explain three (3) common breast complications that develop during puerperium. (6 marks)
43. Pain during labor is a common and significant experience for many women. It can have various effects, both physical and emotional. Explain two (2) effects of each during labour. (4 marks)
44. Using a diagram, illustrate a Sagittal Section of Anatomical relations of the female reproductive system. (5 marks)

45. In the mechanism of normal labor, after descent, the passive movements that facilitate the progress of the baby through the birth canal are many: Explain three (3) of those movement (6 marks)
46. The Active Management of the Third Stage of Labor (AMTSL) is an evidence-based approach used to actively intervene in the process following the birth of the baby and delivery of the placenta. Explain three (3) of the key components (6 marks)
47. Oligohydramnios is a condition characterized by a lower than normal amount of amniotic fluid surrounding the fetus in the uterus during pregnancy. State three (3) causes, and three (3) clinical signs and symptoms of oligohydramnios. (6 marks)

SECTION 3: LONG ANSWER QUESTION (20 MARKS)

48. Madam sue has been admitted in the labour ward in advanced stage of labour. Under your care she has delivered normally but the baby is small for gestational age. Describe the characteristics of a small for gestational age and manage the baby for the first 48 hours (20 marks)